

NIC CPD Micro-Credential: Activities of Daily Living (ADL) Assistance & Personal Care for Professional Caregivers

Issuing organisation: National Institute of Caregivers Nigeria (NIC)

Credential type: Stand-alone Continuing Professional Development (CPD) Micro-Credential

Recommended duration: 5–6 CPD learning hours

Audience: Professional caregivers and direct-care staff

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Central principle: Good personal care is not about doing everything for the client. It is about supporting the person to do as much as they safely can for themselves.

Scope statement: This is professional caregiver CPD. It does not qualify a learner as a nurse, physiotherapist, occupational therapist, medical practitioner or licensed clinical professional. It does not replace specialist moving-and-handling, dysphagia, catheter, wound, medication or clinical-skills training.

1. Course title

NIC CPD Micro-Credential: Activities of Daily Living (ADL) Assistance & Personal Care for Professional Caregivers

2. Course overview

This practical CPD teaches caregivers how to support bathing, showering, grooming, dressing, toileting, continence, oral care, eating, drinking, mobility, transfers, positioning and personal hygiene while protecting dignity, privacy, independence, choice, comfort, safety and cultural preferences.

ADL assistance is a professional competency rather than simply “helping old people.” The caregiver learns to distinguish **assisting** from **taking over** and to adjust support as the client’s strength, pain, cognition, confidence, health and ability change from day to day.

The course reflects person-centred and integrated-care principles, including WHO’s emphasis on coordinated support that maintains or improves older people’s functioning and intrinsic capacity ¹. It also draws on recognised dignity, privacy, mobility and nutrition principles ² ³. Nigerian examples are included, while legal and regulatory

duties must be confirmed against current Nigerian law, employer policy and the learner's role.

Course at a glance

Element	Specification
Audience	Professional caregivers, healthcare assistants, home-care workers, residential-care staff, personal-care assistants and supervisors
Duration	5–6 CPD learning hours
Structure	6 modules, 18 lessons, module assessments and final assessment
Framework	ASSESS → ASK → PLAN → SUPPORT → PROMOTE INDEPENDENCE → MONITOR → DOCUMENT → REVIEW
Online pass mark	80% recommended
Practical pathway	Separate supervised demonstration for physical-care competence
Boundary	Not nursing, therapy, medicine or professional licensure

3. Strategic purpose

Clients often experience personal care as intimate, exposing and potentially disempowering. A rushed or poorly planned intervention can cause falls, burns, skin damage, infection, embarrassment, fear or loss of confidence. A respectful intervention can preserve routine, identity, self-esteem and independence.

This CPD gives working caregivers a consistent way to decide what support is needed, ask what the person wants, plan safely, provide only the assistance required, notice changes, document and review. It is designed for homes, residential settings and community care in Nigeria, where family involvement, shared living spaces, religious practices, water and power availability, language and local routines may affect delivery.

4. Target audience

The target audience is professional caregivers, healthcare assistants, home-care workers, residential-care staff, community-care workers, personal-care assistants, care supervisors and other personnel who provide direct assistance with everyday living activities.

5. Recommended CPD hours

The recommended online learning time is **5–6 CPD learning hours**. If NIC issues a practical competency endorsement, the supervised demonstration and assessment should be separately scheduled and recorded.

6. Prerequisites

Learners should have basic caregiving orientation, communication skills and awareness of workplace reporting and escalation. No clinical qualification is required for the online knowledge pathway. Practical verification requires an appropriate environment, equipment, assessor and local moving-and-handling/safeguarding procedures.

7. Learning outcomes

By the end of the CPD, the learner should be able to:

1. Explain basic ADLs and IADLs and identify role boundaries.
2. Apply person-centred care to everyday assistance.
3. Protect privacy, dignity, consent, choice and cultural preferences.
4. Assess the level of help required without automatically taking over.
5. Prepare and provide safe bathing, showering, bed bathing and grooming support.
6. Assist with oral care, dressing and undressing respectfully.
7. Provide dignified toileting and continence support.
8. Support eating, drinking and hydration while recognising concerns that require escalation.
9. Apply safe principles for mobility, transfers, positioning and environmental preparation.
10. Adapt support for dementia, disability, sensory loss, frailty, pain and communication needs.
11. Respond professionally to resistance, embarrassment and refusal of care.
12. Recognise safeguarding risks associated with intimate care.
13. Observe, document, report and review changes in ADL ability and care outcomes.

14. Demonstrate practical skills only through supervised verification, not online completion alone.

8. Core competency

Provide safe, respectful and person-centred assistance with activities of daily living while promoting independence, dignity, privacy, choice and client safety.

9. ADL competency framework

The NIC A-S-P-S-I-M-D-R framework

ASSESS: Observe ability, environment, risk, comfort, communication and immediate changes.

ASK: Ask what the person wants, what they can do, what help they prefer and what matters culturally or personally.

PLAN: Prepare the environment, equipment, time, privacy and sequence.

SUPPORT: Provide the least intrusive safe assistance needed.

PROMOTE INDEPENDENCE: Encourage participation, pacing, adaptive equipment and choice.

MONITOR: Notice fatigue, pain, dizziness, skin changes, swallowing concerns, distress and changed ability.

DOCUMENT: Record relevant facts, assistance level, changes, concerns and actions.

REVIEW: Discuss what worked, update the plan through authorised channels and escalate when needed.

Levels of assistance

Level	Practical meaning
Independent	Person completes the activity safely without caregiver help
Supervision	Caregiver remains available for safety
Prompting	Verbal, visual or simple cue supports completion
Minimal assistance	Person completes most of the task; caregiver gives small physical help

Moderate assistance	Person completes part of the task; caregiver provides substantial help
Maximal assistance	Person contributes minimally; caregiver provides most help safely
Full assistance	Person cannot safely participate; caregiver completes the task within authorised role

The level may change each day. “I helped yesterday” is not a reason to repeat the same level today.

10. Complete module structure

Module	Title	Lessons	Assessment
1	ADLs, Personhood, Consent and Dignity	1. Understanding ADLs; 2. Person-Centred Care and Choice; 3. Consent, Privacy and Dignity	6 questions
2	Assessing Assistance and Personal Hygiene	4. Assistance Levels and Planning; 5. Bathing, Showering and Bed Bath; 6. Grooming and Oral Care	6 questions
3	Dressing, Toileting and Continence	7. Dressing and Undressing; 8. Toileting; 9. Continence Support and Dignity	6 questions
4	Mealtime Support, Hydration and Mobility	10. Eating and Drinking; 11. Cultural and Religious Preferences; 12. Mobility, Transfers and Positioning	6 questions
5	Adapted Care, Resistance and Safeguarding	13. Dementia, Disability and Sensory Needs; 14. Resistance, Refusal and Distress;	6 questions

		15. Safeguarding in Personal Care	
6	Documentation, Review and Practical Competency	16. Observation and Documentation; 17. Reviewing Care and Escalating Change; 18. Integrated ADL Practice and Practical Pathways	6 questions

11. Detailed lesson outline

Lesson	Purpose
1	Define ADLs/IADLs and professional scope
2	Make care person-centred rather than task-centred
3	Apply consent, privacy and dignity in intimate care
4	Choose the least assistance needed
5	Provide safe bathing and bed-bath support
6	Support grooming and oral care
7	Assist dressing while promoting choice
8	Provide safe, dignified toileting
9	Support continence without shame or clinical overreach
10	Support independent eating, drinking and hydration
11	Respect cultural and religious food practices
12	Apply safe mobility, transfers and positioning boundaries
13	Adapt care for dementia, disability and sensory needs
14	Respond to refusal, resistance and distress

15	Recognise intimate-care safeguarding risks
16	Document observations and assistance accurately
17	Review change, escalate and coordinate with other NIC courses
18	Integrate the framework and distinguish knowledge from practice

12. Full lesson content

Module 1 — ADLs, Personhood, Consent and Dignity

Lesson 1. Understanding ADLs

Why this matters

ADLs are everyday activities needed for personal functioning and wellbeing. Supporting them is central to professional caregiving.

Core content

Basic ADLs include bathing, showering, grooming, dressing, toileting, continence, oral care, eating, drinking, mobility, transfers, positioning and hygiene. IADLs are more complex activities such as meal preparation, shopping, housekeeping, communication, transportation, appointments and medication-related support where authorised.

The exact role depends on training, employer, care plan and applicable requirements. Medication administration, catheter care, wound care, specialist swallowing management, clinical assessment and therapy are not automatically included in ordinary ADL assistance.

Scenario

A caregiver is asked to change a catheter while helping with toileting.

Best response: Explain that catheter care requires specific training and authorisation, maintain dignity and escalate to the appropriate professional.

Common mistakes

Calling every task “personal care,” assuming family instructions override the care plan and drifting into nursing or therapy.

Lesson assessment

1. **MCQ:** Which is a basic ADL? A) Shopping; B) Bathing; C) Appointment management; D) Transport planning. **Answer: B. LO: 1.**
2. **Scenario:** Asked to perform an unauthorised clinical task. **Answer:** Do not perform it; escalate. **LO: 1, 14.**
3. **True/False:** IADLs can include meal preparation and housekeeping. **Answer: True. LO: 1.**
4. **What should not happen?** Treat ADL care as unskilled work. **Answer:** Recognise it as professional competency. **LO: 1.**
5. **Application:** Name four basic ADLs. **Answer:** Bathing, dressing, toileting, eating, drinking, grooming or mobility. **LO: 1.**
6. **Scope:** Does a caregiver automatically have authority for every task? **Answer: No. LO: 1.**

Lesson 2. Person-Centred Care and Choice

Why this matters

“Care for me” can unnecessarily replace ability. “Care with me” supports participation and personhood.

Core content

Ask: What can the person do? What help is wanted? What routines matter? What cultural or religious practices matter? What causes discomfort? What promotes dignity? Offer realistic choices about timing, clothing, products, sequence, gender of caregiver where possible and family involvement.

Person-centred care is compatible with safety. Choice is not a reason to ignore a serious risk; it is a reason to explain the risk, seek agreement, use the least restrictive safe option and escalate unresolved concerns.

Scenario

A client can wash their face but the caregiver completes it because the shift is busy.

Best response: Allow participation and provide only the support needed, while managing time professionally.

Common mistakes

Rushing, speaking about the person as if absent, imposing routines and treating preference as inconvenience.

Lesson assessment

1. **MCQ:** “Care with me” means: A) doing everything; B) supporting participation; C) ignoring risk; D) refusing help. **Answer: B. LO: 2.**
2. **Scenario:** Client wants a familiar cultural routine. **Answer:** Respect it where safe and consistent with the plan. **LO: 2.**
3. **True/False:** Person-centred care can include choice about timing. **Answer: True. LO: 2.**
4. **What should not happen?** Impose the caregiver’s preferred routine without discussion. **LO: 2.**
5. **Application:** Name four questions to ask. **Answer:** What can you do? What help do you want? What matters? What is uncomfortable? **LO: 2.**
6. **Scenario:** Preference creates immediate serious risk. **Answer:** Explain, reduce risk and escalate; do not silently ignore it. **LO: 2, 14.**

Lesson 3. Consent, Privacy and Dignity

Why this matters

Personal care involves the body and private information. Consent and dignity are not optional courtesies.

Core content

Knock before entering. Introduce yourself. Explain what will happen. Ask permission before assistance and before touching. Close doors and curtains. Keep the body covered except the area being cared for. Use respectful language, allow time, protect belongings and avoid infantilising adults.

Respect refusal. Explore the reason, offer alternatives and report an unresolved safety concern. Communication difficulty does not mean automatic inability to decide. Use accessible communication and escalate concerns about capacity through the proper pathway. Being someone’s caregiver does not automatically give permission to do things to their body.

Scenario

A client refuses a shower but agrees to a wash at the basin.

Best response: Respect the choice, support the agreed alternative and document/report if the refusal creates an unresolved risk.

Common mistakes

Exposing the person unnecessarily, discussing care in public, assuming silence means consent and threatening or shaming.

Lesson assessment

1. **MCQ:** Before intimate care, the caregiver should: A) begin; B) explain and seek consent; C) call friends; D) remove all clothing. **Answer: B. LO: 3.**
2. **Scenario:** Client refuses shower. **Answer:** Respect, explore, offer safe alternatives and escalate risk if needed. **LO: 3.**
3. **True/False:** Silence automatically means consent. **Answer: False. LO: 3.**
4. **What should not happen?** Leave doors open during intimate care. **LO: 3.**
5. **Application:** Name five dignity actions. **Answer:** Knock, explain, consent, cover, privacy, respectful language and time. **LO: 3.**
6. **Scope:** What if capacity is uncertain? **Answer:** Support communication and escalate through authorised channels; do not decide alone. **LO: 3.**

Module 2 — Assessing Assistance and Personal Hygiene

Lesson 4. Assistance Levels and Planning

Why this matters

The right level of help protects independence and safety.

Core content

Observe before helping. Consider alertness, balance, pain, strength, range of movement, fatigue, environment, communication and known risks. Start with the least assistance that is safe: independent, supervision, prompting, minimal, moderate, maximal or full assistance.

Plan time, equipment, privacy, temperature, infection-prevention measures and an exit route. Do not pull, lift or improvise if a moving-and-handling procedure is required. Use the care plan and seek trained assistance.

Scenario

A client who usually needs moderate help is unusually weak and dizzy.

Best response: Pause, keep safe, do not proceed as usual, report the change and seek appropriate assessment.

Common mistakes

Assuming yesterday's ability, rushing, using furniture as a lifting aid and failing to report changed ability.

Lesson assessment

1. **MCQ:** The least intrusive safe support is preferred because it: A) saves all time; B) promotes ability and dignity; C) avoids observation; D) removes choice. **Answer: B. LO:** 4.
2. **Scenario:** New dizziness during care. **Answer:** Stop/keep safe, monitor and escalate. **LO:** 4, 13.
3. **True/False:** Assistance level may change day to day. **Answer: True. LO:** 4.
4. **What should not happen?** Pull a client by the arms. **LO:** 4, 9.
5. **Application:** Name six planning factors. **Answer:** Ability, risk, equipment, privacy, time, environment, communication and infection prevention. **LO:** 4.
6. **Scenario:** No safe equipment for transfer. **Answer:** Do not improvise; seek trained help. **LO:** 4, 9.

Lesson 5. Bathing, Showering and Bed Bath

Why this matters

Water, heat, wet floors and intimate exposure create safety and dignity risks.

Core content

Prepare products, towels, clothing, non-slip support, privacy and a safe temperature check according to local procedure. Explain the sequence. Encourage the client to wash areas they can safely reach. Maintain warmth and cover areas not being washed. Monitor for dizziness, weakness, pain, breathlessness or distress.

For bed bathing, prepare materials, position safely, sequence from cleaner to less clean areas, change water/materials appropriately, prevent chilling and observe skin. Do not prescribe wound treatment. Report redness, broken skin, swelling, unusual pain, bruising, pressure concerns or other changes.

Scenario

A client becomes dizzy in the shower.

Best response: Stop, keep the person safe, provide support without unsafe lifting, call assistance and follow the emergency/escalation procedure.

Common mistakes

Hot water, wet floors, leaving the client exposed, reusing contaminated water/materials and treating skin changes independently.

Lesson assessment

1. **MCQ:** A shower preparation priority is: A) privacy and non-slip safety; B) speed only; C) maximum heat; D) leaving the door open. **Answer: A. LO: 5.**
2. **Scenario:** Dizziness in shower. **Answer:** Stop, protect, call assistance and escalate. **LO: 5.**
3. **True/False:** Skin observations should be reported, not diagnosed by the caregiver. **Answer: True. LO: 5.**
4. **What should not happen?** Leave the client unnecessarily exposed. **LO: 5.**
5. **Application:** Name five bathing risks. **Answer:** Falls, scalds, skin damage, infection and loss of dignity. **LO: 5.**
6. **Scenario:** Red area over pressure point. **Answer:** Record and report promptly; do not prescribe treatment. **LO: 5, 13.**

Lesson 6. Grooming and Oral Care

Why this matters

Grooming supports identity, confidence, culture and comfort. Oral care affects comfort and participation in eating and communication.

Core content

Ask about preferred hairstyle, shaving, products, clothing presentation and religious/cultural practice. Support hair care, shaving, skincare, nail-care awareness and oral hygiene within role. Do not cut nails or perform procedures when circulation, diabetes, infection, anticoagulation, deformity or policy makes specialist input necessary.

For oral care, position safely, provide toothbrush/toothpaste or denture support as appropriate, clean equipment and handle dentures carefully. Report bleeding, sores,

swelling, unusual pain, persistent bad breath, difficulty swallowing or damaged dentures. Do not diagnose oral disease.

Scenario

A client has bleeding gums and pain but asks the caregiver to continue brushing forcefully.

Best response: Provide gentle support within the plan, stop if harmful and report the concern for professional review.

Common mistakes

Using products without checking preference, damaging dentures, sharing equipment and ignoring oral changes.

Lesson assessment

1. **MCQ:** Grooming can support: A) identity and confidence; B) only appearance; C) diagnosis; D) punishment. **Answer: A. LO: 6.**
2. **Scenario:** Damaged dentures. **Answer:** Protect them and report for appropriate review. **LO: 6.**
3. **True/False:** Caregivers should diagnose oral disease. **Answer: False. LO: 6.**
4. **What should not happen?** Share oral-care equipment. **LO: 6.**
5. **Application:** Name four oral concerns to report. **Answer:** Bleeding, sores, swelling, pain, swallowing difficulty, bad breath or damaged dentures. **LO: 6.**
6. **Scenario:** Nail care may be clinically risky. **Answer:** Follow policy and seek specialist direction. **LO: 6.**

Module 3 — Dressing, Toileting and Continence

Lesson 7. Dressing and Undressing

Why this matters

Clothing expresses identity and can either support or remove autonomy.

Core content

Offer choices appropriate to weather, routine, culture, religion and the person's abilities. Maintain warmth and privacy. Encourage the client to choose, reach, fasten and sequence as able. For one-sided weakness or limited mobility, use the authorised technique taught by the employer or moving-and-handling instructor; do not improvise.

Use visual, verbal or tactile prompts according to consent. Avoid pulling limbs or forcing joints. Report new weakness, pain, swelling, confusion or inability to complete a familiar step.

Scenario

A client can select clothes and put one arm through a sleeve but needs help with fasteners.

Best response: Let the client choose and complete what they can; provide minimal assistance with fasteners.

Common mistakes

Choosing clothing without asking, rushing, exposing the person and doing every step for convenience.

Lesson assessment

1. **MCQ:** Dressing support should promote: A) choice and participation; B) speed only; C) caregiver preference; D) exposure. **Answer: A. LO: 7.**
2. **Scenario:** Client needs help only with buttons. **Answer:** Provide minimal targeted assistance. **LO: 7.**
3. **True/False:** Clothing may express culture and identity. **Answer: True. LO: 7.**
4. **What should not happen?** Force a painful limb into a sleeve. **LO: 7.**
5. **Application:** Name four adaptations. **Answer:** Extra time, prompts, easy fasteners, privacy, warmth and accessible layout. **LO: 7.**
6. **Scenario:** New weakness during dressing. **Answer:** Stop as needed, keep safe and report. **LO: 7, 13.**

Lesson 8. Toileting

Why this matters

Toileting is intimate, time-sensitive and closely linked to dignity and safety.

Core content

Provide safe access, appropriate lighting, clear routes, privacy, communication and enough time. Ask how the person prefers assistance. Ensure the call bell or communication method is accessible. Support hygiene without unnecessary exposure. Monitor urgency, repeated difficulty, pain, confusion, falls risk and changes in pattern.

Never mock, complain, display disgust or discuss toileting unnecessarily. Follow the care plan and seek help for transfers or equipment beyond training.

Scenario

A client repeatedly asks for the toilet and appears confused.

Best response: Respond respectfully, keep safe, observe and report the change rather than labelling the person difficult.

Common mistakes

Rushing, leaving the person unsupported, blocking access, infantilising language and ignoring repeated changes.

Lesson assessment

1. **MCQ:** Toileting assistance should prioritise: A) privacy and safe access; B) speed over dignity; C) public discussion; D) refusal. **Answer: A. LO: 8.**
2. **Scenario:** Repeated toileting difficulty. **Answer:** Provide respectful support and report the change. **LO: 8, 13.**
3. **True/False:** Toileting concerns may require documentation and escalation. **Answer: True. LO: 8.**
4. **What should not happen?** Treat an adult like a child. **LO: 8.**
5. **Application:** Name five safety factors. **Answer:** Lighting, access, clear route, equipment, call method, privacy and time. **LO: 8.**
6. **Scenario:** Client cannot transfer safely. **Answer:** Do not improvise; seek trained assistance. **LO: 8, 12.**

Lesson 9. Continence Support and Dignity

Why this matters

Incontinence can cause shame, isolation, skin risk and loss of confidence. Professional support protects dignity.

Core content

Continence support may involve urinary or bowel continence, products, hygiene, clothing changes, skin-protection awareness, hydration awareness and reporting changes. Follow the care plan and product instructions. Change privately, dispose of waste appropriately and protect belongings.

Report new incontinence, blood, unusual pain, feverish illness, strong change in smell or colour, constipation/diarrhoea, skin breakdown, repeated accidents or distress. Do not prescribe treatment or insert devices unless separately trained and authorised.

Scenario

A client has an accident and says, “Please do not tell anyone.”

Best response: Reassure without promising secrecy, provide private support and document/report relevant concerns through the proper pathway.

Common mistakes

Mocking, visible disgust, discussing the accident, delaying hygiene and promising absolute secrecy.

Lesson assessment

1. **MCQ:** The best response to incontinence is: A) shame; B) discreet, respectful support; C) complaint; D) public discussion. **Answer: B. LO: 9.**
2. **Scenario:** New skin breakdown. **Answer:** Protect dignity and report promptly; do not prescribe. **LO: 9.**
3. **True/False:** New continence changes should be reported. **Answer: True. LO: 9.**
4. **What should not happen?** Display disgust. **LO: 9.**
5. **Application:** Name four reportable changes. **Answer:** Blood, pain, skin breakdown, new pattern, diarrhoea, constipation or distress. **LO: 9.**
6. **Scenario:** Client asks for secrecy about a safeguarding concern. **Answer:** Be supportive but follow reporting duties. **LO: 9, 15.**

Module 4 — Mealtime Support, Hydration and Mobility

Lesson 10. Eating, Drinking and Hydration

Why this matters

Mealtimes support nutrition, hydration, comfort, culture and social connection. Unsafe assistance can cause choking or distress.

Core content

Prepare a clean, calm environment. Perform hand hygiene. Position the person as specified in the care plan. Offer adaptive utensils, appropriate pace, manageable portions and

preferred drinks where authorised. Encourage self-feeding and never force food or drink. Report coughing during meals, choking, difficulty swallowing, wet/gurgly voice, sudden refusal, repeated poor intake, fatigue or weight-related concerns. Do not diagnose dysphagia or change food texture without specialist instruction. Never apply a universal fluid target: follow the person's plan, including restrictions.

Scenario

A client coughs repeatedly while drinking.

Best response: Pause, keep the person safe, follow the care plan, report promptly and seek specialist guidance; do not independently thicken fluids.

Common mistakes

Force-feeding, rushing, lying the person flat, changing texture without authority and ignoring repeated coughing.

Lesson assessment

1. **MCQ:** Mealtime support should include: A) pacing and safe positioning; B) forcing; C) rushing; D) lying flat. **Answer: A. LO: 10.**
2. **Scenario:** Repeated coughing while drinking. **Answer:** Pause, follow plan and escalate; do not diagnose or modify texture. **LO: 10.**
3. **True/False:** A universal fluid target suits every client. **Answer: False. LO: 10.**
4. **What should not happen?** Force food or drink. **LO: 10.**
5. **Application:** Name five mealtime concerns to report. **Answer:** Coughing, choking, swallowing difficulty, refusal, poor intake, fatigue or voice change. **LO: 10.**
6. **Scenario:** Client wants an adaptive utensil. **Answer:** Support it if safe and authorised. **LO: 10.**

Lesson 11. Cultural and Religious Preferences

Why this matters

Food and personal-care routines can carry identity, faith, family and community meaning.

Core content

Ask about cultural foods, fasting, religious practices, family traditions, food taboos and personal preferences. Respect choice while following authorised dietary restrictions and

care plans. Do not ridicule unfamiliar foods or assume a family member's preference is the client's preference.

When fasting or a cultural practice may conflict with medication, hydration, diabetes, swallowing or another risk, explain respectfully and escalate to the appropriate professional. The caregiver should not make the clinical decision.

Scenario

A client is fasting and has a documented hydration concern.

Best response: Respect the practice, explain the concern, follow the authorised plan and escalate for coordinated advice.

Common mistakes

Mocking food, overriding faith practices, giving unapproved food and keeping no record of preferences.

Lesson assessment

1. **MCQ:** Cultural preference should be: A) ignored; B) respected within the care plan and safety requirements; C) mocked; D) replaced automatically. **Answer: B. LO: 11.**
2. **Scenario:** Fasting may conflict with care plan. **Answer:** Respectfully escalate; do not decide clinically alone. **LO: 11.**
3. **True/False:** Family tradition automatically overrides client preference. **Answer: False. LO: 11.**
4. **What should not happen?** Ridicule food or religious practice. **LO: 11.**
5. **Application:** Name four preference areas. **Answer:** Food, fasting, family practice, taboo, timing, faith and personal choice. **LO: 11.**
6. **Scenario:** Preferred food conflicts with restriction. **Answer:** Follow authorised restriction and seek review. **LO: 11.**

Lesson 12. Mobility, Transfers and Positioning

Why this matters

Falls and musculoskeletal injuries can harm both client and caregiver. Safe support requires assessment, equipment and training.

Core content

Prepare the route, footwear, clothing, equipment, brakes, surfaces and communication. Explain each step. Use the person's care plan and approved moving-and-handling technique. Do not manually lift beyond training, pull by limbs or improvise with unsafe furniture. If the person becomes dizzy, weak or distressed, stop and seek help.

Positioning should support comfort, breathing, skin protection and function according to the plan. Specialist transfer, hoist, spinal, post-operative or neurological techniques require appropriate training. Link learners to NIC's Mobility, Transfers & Safe Patient Handling CPD rather than duplicating it.

Scenario

A client slips while standing and the caregiver cannot safely support them.

Best response: Protect from further harm, call trained assistance and do not attempt an unsafe lift.

Common mistakes

Twisting, lifting alone, failing to check brakes, rushing and using a technique not taught.

Lesson assessment

1. **MCQ:** Before transfer, check: A) route and equipment; B) nothing; C) only speed; D) the caregiver's convenience. **Answer: A. LO: 12.**
2. **Scenario:** Client becomes dizzy standing. **Answer:** Stop, keep safe and call assistance. **LO: 12.**
3. **True/False:** A caregiver should pull a client by the arms. **Answer: False. LO: 12.**
4. **What should not happen?** Improvise a hoist technique. **LO: 12.**
5. **Application:** Name six checks. **Answer:** Route, footwear, clothing, brakes, equipment, communication, balance and ability. **LO: 12.**
6. **Scenario:** Complex post-operative transfer. **Answer:** Follow specialist plan and trained procedure; do not improvise. **LO: 12.**

Module 5 — Adapted Care, Resistance and Safeguarding

Lesson 13. Dementia, Disability and Sensory Needs

Why this matters

A diagnosis or disability does not remove personhood. Adaptation supports communication, safety and participation.

Core content

Use the person's preferred communication, short explanations, visual cues, familiar routines, adequate lighting and reduced distraction. Allow extra time. Consider hearing, vision, language, cognition, pain, fatigue, trauma and cultural needs. Ask before using touch.

For dementia, do not assume resistance means deliberate non-compliance. Look for pain, fear, embarrassment, unfamiliarity, fatigue, infection or environmental stress and follow the person's care plan. Report sudden changes.

Scenario

A client with dementia becomes distressed when a new caregiver approaches with a towel.

Best response: Pause, introduce yourself, explain, reduce stimulation, offer choice and try again later or seek guidance.

Common mistakes

Arguing, shouting, infantilising, rushing, removing aids and treating behaviour as a character flaw.

Lesson assessment

1. **MCQ:** Adapted care may include: A) extra time and visual cues; B) shouting; C) removing hearing aids; D) forced care. **Answer: A. LO: 13.**
2. **Scenario:** Distress during personal care. **Answer:** Pause, reduce triggers, communicate and reassess. **LO: 13–14.**
3. **True/False:** Pain or fear may affect cooperation. **Answer: True. LO: 13.**
4. **What should not happen?** Argue with a confused client. **LO: 13.**
5. **Application:** Name five adaptations. **Answer:** Short explanations, visual cues, familiar routine, lighting, reduced noise, extra time and accessible communication. **LO: 13.**
6. **Scenario:** Sudden new confusion. **Answer:** Keep safe and report promptly. **LO: 13, 17.**

Lesson 14. Resistance, Refusal and Distress

Why this matters

Refusal may communicate fear, pain, shame, tiredness, confusion, loss of control or a reasonable preference.

Core content

Pause and listen. Use calm language and preserve privacy. Ask what is wrong, offer choice, adjust timing, involve a trusted person with consent and try a different approach. Do not threaten, restrain, shame or force ordinary personal care. If an urgent safety risk remains, escalate according to policy.

Document what happened objectively: what was offered, the person's response, possible triggers, alternatives offered, outcome and who was informed.

Scenario

A client refuses continence care after a previous humiliating experience.

Best response: Acknowledge distress, offer control and privacy, explain options, involve authorised support and document/escalate if risk remains.

Common mistakes

Calling refusal “attention-seeking,” forcing care, promising secrecy or omitting the event from documentation.

Lesson assessment

1. **MCQ:** Refusal should first be met with: A) force; B) listening and exploration; C) ridicule; D) abandonment. **Answer: B. LO: 14.**
2. **Scenario:** Client refuses intimate care. **Answer:** Pause, offer choices and escalate unresolved risk. **LO: 14.**
3. **True/False:** A caregiver may restrain someone for ordinary personal-care convenience. **Answer: False. LO: 14–15.**
4. **What should not happen?** Label the person difficult. **LO: 14.**
5. **Application:** Name five possible triggers. **Answer:** Pain, fear, shame, fatigue, confusion, unfamiliarity or loss of control. **LO: 14.**
6. **Scenario:** Risk remains after refusal. **Answer:** Follow escalation/safeguarding procedure. **LO: 14–15.**

Lesson 15. Safeguarding in Personal Care

Why this matters

Intimate care creates opportunities for neglect, humiliation, coercion, financial abuse, sexual abuse, physical abuse and boundary violations.

Core content

Warning signs may include unexplained injuries, fear of a caregiver, sudden withdrawal, missing belongings, repeated unexplained skin problems, poor hygiene, unexplained distress during care, sexualised behaviour inconsistent with context, or a caregiver insisting on secrecy or unnecessary exposure.

Maintain professional boundaries: no sexualised language, unnecessary touching, private gifts, personal loans, secret communication, social-media images or unauthorised disclosure. Use two-person procedures where policy requires. Report concerns through safeguarding channels. Do not investigate, confront an alleged abuser or promise confidentiality.

Scenario

A caregiver notices a colleague closes the curtain, refuses observation and repeatedly makes sexual jokes during intimate care.

Best response: Protect the client, record facts and report through the safeguarding route; do not confront alone or investigate.

Common mistakes

Normalising boundary violations, blaming the client, delaying report and discussing allegations publicly.

Lesson assessment

1. **MCQ:** A safeguarding concern should be: A) ignored; B) reported through the proper route; C) investigated by the caregiver; D) posted online. **Answer: B. LO: 15.**
2. **Scenario:** Unnecessary exposure by colleague. **Answer:** Protect, record and report. **LO: 15.**
3. **True/False:** Caregivers should promise absolute secrecy. **Answer: False. LO: 15.**
4. **What should not happen?** Confront an alleged abuser alone. **LO: 15.**
5. **Application:** Name four boundary risks. **Answer:** Sexualised jokes/touch, gifts/loans, secret communication, images or unnecessary disclosure. **LO: 15.**
6. **Scenario:** Client discloses harm. **Answer:** Listen calmly, do not interrogate, explain limits of confidentiality and report. **LO: 15.**

Module 6 — Documentation, Review and Practical Competency

Lesson 16. Observation and Documentation

Why this matters

Accurate records support continuity, safety, accountability and review.

Core content

Record date/time, activity, level of assistance, participation, relevant observations, refusal or preference, changes, action taken, report/escalation and follow-up. Use objective language: “required two prompts and minimal help to fasten shirt,” not “was lazy.” Record direct statements accurately where relevant.

Do not diagnose, exaggerate, omit incidents, share images or leave records accessible. Follow approved paper/digital systems and confidentiality requirements.

Scenario

A caregiver writes, “Client was uncooperative and dirty.”

Best response: Replace judgement with observable facts, such as the care offered, response, hygiene support provided and any concern reported.

Lesson assessment

1. **MCQ:** Objective documentation records: A) judgement; B) observed facts and actions; C) gossip; D) blame. **Answer: B. LO: 16.**
2. **Scenario:** Client completes more than usual. **Answer:** Record changed assistance level and participation. **LO: 16.**
3. **True/False:** A refusal may be relevant to document. **Answer: True. LO: 16.**
4. **What should not happen?** Share a personal-care photo without authority. **LO: 16.**
5. **Application:** Name six documentation fields. **Answer:** Time, activity, assistance, participation, observations, action, escalation and follow-up. **LO: 16.**
6. **Scenario:** Record system unavailable. **Answer:** Use approved downtime process and enter accurately later. **LO: 16.**

Lesson 17. Reviewing Care and Escalating Change

Why this matters

ADL changes may signal pain, illness, injury, delirium, depression, medication effects or environmental problems. The caregiver does not diagnose but must notice and report.

Core content

Review whether the plan still matches ability, preference, safety and dignity. Escalate sudden decline, new weakness, falls, dizziness, pain, confusion, swallowing concern, repeated refusal, skin breakdown, continence change, poor intake, breathlessness or distress.

Use the appropriate professional pathway. Link to NIC' s First Aid & BLS, Elderly Care & Gerontology, Home Health & Chronic Care, Dementia Care, Infection Prevention & Control, Mobility/Transfers, Nutrition/Hydration and Care Documentation CPDs. Introduce rather than duplicate specialist content.

Scenario

A client who normally dresses independently now needs full assistance and appears drowsy.

Best response: Keep safe, report the sudden change urgently through the appropriate pathway and document facts.

Lesson assessment

1. **MCQ:** Sudden ADL decline should be: A) ignored; B) observed, documented and escalated; C) blamed; D) hidden. **Answer: B. LO: 17.**
2. **Scenario:** New drowsiness and weakness. **Answer:** Urgent escalation. **LO: 17.**
3. **True/False:** Caregivers diagnose the reason for every change. **Answer: False. LO: 17.**
4. **What should not happen?** Continue unsafe routine without reporting. **LO: 17.**
5. **Application:** Name six escalation triggers. **Answer:** Falls, pain, confusion, swallowing concern, skin breakdown, continence change, poor intake or breathlessness. **LO: 17.**
6. **Scenario:** Need deeper transfer training. **Answer:** Refer to appropriate NIC specialist CPD/employer training. **LO: 17.**

Lesson 18. Integrated ADL Practice and Practical Pathways

Why this matters

Professional personal care combines technical preparation with dignity, choice, observation, documentation and review.

Core content

Apply the full framework: assess ability and risk, ask preference and consent, plan environment and equipment, support the task, promote independence, monitor changes,

document and review. The online course demonstrates knowledge and judgement. Practical skills require observed performance.

An integrated practical station may include entering a room, explaining a bed bath, preparing equipment, maintaining privacy, supporting participation, observing skin, cleaning equipment, documenting assistance and reporting a change. Transfers and specialist tasks should be assessed only within the assessor's competence and the learner's training.

Scenario

A client with one-sided weakness wants to shower independently but is dizzy and the bathroom floor is wet.

Best response: Explain the risk, pause the activity, make the environment safe, seek assistance and preserve choice through a safer alternative rather than taking over silently.

Lesson assessment

1. **Application:** List the framework. **Answer:** Assess, Ask, Plan, Support, Promote Independence, Monitor, Document, Review. **LO:** 18.
2. **Scenario:** Wet floor and dizziness. **Answer:** Stop, make safe, seek help and offer safer options. **LO:** 18.
3. **MCQ:** Practical competency requires: A) reading only; B) observed demonstration; C) attendance only; D) self-declaration. **Answer:** B. **LO:** 14.
4. **What should not happen?** Take over without explanation. **LO:** 18.
5. **Application:** Name five practical station behaviours. **Answer:** Consent, privacy, preparation, safe support, independence, observation, cleaning and documentation. **LO:** 18.
6. **Scenario:** Learner cannot perform a transfer safely. **Answer:** Stop, seek help and arrange proper training/remediation. **LO:** 12, 18.

13. Lesson assessments

Each lesson includes six structured items with immediate feedback. The online assessments test knowledge, judgement, dignity, consent, safety, scope and escalation. Practical physical tasks are not signed off through online quizzes.

14. Module assessments

Module 1

1. Define ADL and IADL. **Answer:** Basic personal functioning versus more complex household/community tasks.
2. Scenario: client chooses a preferred routine. **Answer:** Respect where safe and authorised.
3. What should not happen? **Answer:** Begin intimate care without explanation/consent.
4. True/False: Silence automatically means consent. **Answer:** False.
5. Application: list dignity actions. **Answer:** Knock, explain, cover, privacy, respectful language and time.
6. Scope scenario: unauthorised clinical task. **Answer:** Decline and escalate.

Module 2

1. Scenario: changed ability. **Answer:** Reassess and report.
2. Bathing MCQ. **Answer:** Non-slip preparation and privacy.
3. Oral-care concern. **Answer:** Report, do not diagnose.
4. What should not happen? **Answer:** Force a painful limb or expose unnecessarily.
5. Bed-bath application. **Answer:** Warmth, sequencing, clean materials, skin observation.
6. Assistance-level question. **Answer:** Use least safe assistance.

Module 3

1. Dressing scenario. **Answer:** Support choice and targeted help.
2. Toileting MCQ. **Answer:** Safe access and privacy.
3. Continence concern. **Answer:** Report new change/skin breakdown.
4. What should not happen? **Answer:** Mock or display disgust.
5. Refusal scenario. **Answer:** Explore and offer alternatives.
6. Transfer boundary. **Answer:** Do not improvise.

Module 4

1. Coughing during meal. **Answer:** Pause and escalate.
2. Hydration question. **Answer:** Follow care plan; no universal target.
3. Cultural fasting scenario. **Answer:** Respectfully escalate conflict with plan.
4. Mobility safety. **Answer:** Check route/equipment and use trained technique.
5. What should not happen? **Answer:** Force food/drink or pull limbs.

6. Positioning question. **Answer:** Follow authorised plan.

Module 5

1. Dementia distress scenario. **Answer:** Pause, reduce triggers, offer choice.
2. Disability adaptation. **Answer:** Accessible communication and extra time.
3. Safeguarding MCQ. **Answer:** Report through proper route.
4. What should not happen? **Answer:** Restrain for convenience or investigate.
5. Refusal question. **Answer:** Listen, adapt and escalate risk.
6. Boundary scenario. **Answer:** Protect, record and report.

Module 6

1. Documentation question. **Answer:** Observable facts, assistance and actions.
2. Sudden decline scenario. **Answer:** Escalate.
3. Practical pathway MCQ. **Answer:** Observed demonstration.
4. What should not happen? **Answer:** Claim therapy/nursing competence.
5. Integrated application. **Answer:** Use ASPSIMDR.
6. Review question. **Answer:** Update through authorised channels.

15. Final assessment blueprint

The final assessment contains **35 questions** with a recommended pass mark of **80% (28/35)**. **28 questions are scenario/application based.**

Domain	Questions	Number	Scenario/application
ADLs, personhood and scope	1–5	5	4
Consent, dignity and assistance levels	6–10	5	4
Bathing, grooming, oral care and dressing	11–15	5	4
Toileting, continence, meals and hydration	16–20	5	4

Mobility, adaptation, refusal and safeguarding	21–25	5	4
Documentation, review and escalation	26–30	5	4
Integrated practice and credential limits	31–35	5	4
Total		35	28

16. Final assessment questions

1. **MCQ:** ADLs primarily refer to: A) everyday personal functioning; B) only employment; C) prescribing; D) diagnosis. **Answer: A.**
2. **Scenario:** Asked to perform catheter care without training. **Answer:** Do not perform; escalate. **LO:** 1.
3. **True/False:** ADL support is a professional competency. **Answer: True.**
4. **Scenario:** Client can wash face independently. **Answer:** Encourage and support rather than take over. **LO:** 2.
5. **What should not happen?** Treat every task as caregiver-authorized. **Answer:** Follow role, plan and training. **LO:** 1.
6. **MCQ:** Before intimate care, the caregiver should: A) start; B) explain and seek consent; C) expose fully; D) call visitors. **Answer: B.**
7. **Scenario:** Client refuses shower but accepts basin wash. **Answer:** Respect and support alternative. **LO:** 3.
8. **True/False:** Silence automatically means consent. **Answer: False.**
9. **Scenario:** Client needs only help with buttons. **Answer:** Provide minimal targeted assistance. **LO:** 4, 7.
10. **What should not happen?** Pull a painful limb. **Answer:** Stop and seek appropriate help. **LO:** 4.
11. **MCQ:** Bathing safety includes: A) non-slip precautions and privacy; B) hottest water; C) open doors; D) rushing. **Answer: A.**
12. **Scenario:** Dizziness in shower. **Answer:** Stop, protect and escalate. **LO:** 5.
13. **True/False:** Caregivers diagnose oral disease. **Answer: False.**

14. **Scenario:** Damaged dentures. **Answer:** Protect and report. **LO:** 6.
15. **What should not happen?** Force a joint into clothing. **Answer:** Use safe adapted support. **LO:** 7.
16. **MCQ:** Toileting support should prioritise: A) privacy and safe access; B) public discussion; C) speed only; D) shame. **Answer: A.**
17. **Scenario:** New skin breakdown after incontinence. **Answer:** Report promptly and do not prescribe. **LO:** 9.
18. **True/False:** Mocking incontinence is unprofessional. **Answer: True.**
19. **Scenario:** Repeated coughing while drinking. **Answer:** Pause, follow plan and escalate. **LO:** 10.
20. **What should not happen?** Use a universal fluid target or alter texture independently. **Answer:** Follow authorised plan. **LO:** 10.
21. **Scenario:** Client is fasting with a care-plan conflict. **Answer:** Respectfully escalate. **LO:** 11.
22. **MCQ:** Before transfer, check: A) route, equipment and ability; B) nothing; C) speed only; D) convenience. **Answer: A.**
23. **Scenario:** Client becomes dizzy standing. **Answer:** Stop, keep safe and call assistance. **LO:** 12.
24. **True/False:** Dementia distress may reflect pain or fear. **Answer: True.**
25. **Scenario:** Colleague unnecessarily exposes client. **Answer:** Protect, document and report safeguarding concern. **LO:** 15.
26. **MCQ:** Objective documentation records: A) facts and actions; B) insults; C) gossip; D) assumptions. **Answer: A.**
27. **Scenario:** Sudden full-assistance need and drowsiness. **Answer:** Urgent escalation. **LO:** 17.
28. **True/False:** Refusal may be relevant to document. **Answer: True.**
29. **Scenario:** System unavailable. **Answer:** Use approved downtime process. **LO:** 16.
30. **What should not happen?** Share a personal-care photograph without authority. **Answer:** Protect confidentiality. **LO:** 16.
31. **Integrated scenario:** Wet bathroom, dizziness and desire for independence. **Answer:** Stop, make safe, explain, offer safer choice and seek help. **LO:** 18.
32. **MCQ:** Practical competency requires: A) online reading; B) observed demonstration; C) attendance only; D) self-declaration. **Answer: B.**

33. **Scenario:** Learner cannot complete safe transfer. **Answer:** Stop and arrange proper training/remediation. **LO:** 12, 18.
34. **True/False:** NIC ADL CPD makes a learner a nurse or therapist. **Answer:** False.
35. **Integrated scenario:** Client refuses intimate care after distress, has new confusion and skin concern. **Answer:** Preserve dignity, pause, assess safely, offer choice, report changes and safeguarding risk, document and review. **LO:** 3, 13–18.

17. Answer key and explanations

Q	Answer	Explanation
1	A	ADLs are everyday personal-function activities.
2	Decline and escalate	Clinical tasks require training and authority.
3	True	ADL assistance requires judgement, safety and dignity.
4	Encourage participation	Support should not unnecessarily replace ability.
5	Follow scope	Role and plan define responsibility.
6	B	Consent and explanation protect autonomy.
7	Respect alternative	Refusal should be met with choice and safe options.
8	False	Consent must be appropriately communicated.
9	Minimal help	Use the least assistance that is safe.
10	Stop and seek help	Pain may signal risk.
11	A	Wet environments create fall risk; privacy remains essential.
12	Stop/protect/escalate	Dizziness can precede a fall or reflect illness.
13	False	Observe and report; do not diagnose.

14	Protect/report	Damaged dentures need appropriate review.
15	Safe adaptation	Never force painful movement.
16	A	Toileting is intimate and safety-sensitive.
17	Report	Skin breakdown may require professional assessment.
18	True	Mockery causes harm and violates dignity.
19	Pause/escalate	Coughing may indicate swallowing concern.
20	Follow plan	Fluid and texture needs are individual.
21	Escalate respectfully	Caregiver does not make clinical decisions alone.
22	A	Route, equipment and ability prevent avoidable injury.
23	Stop/call	Unsafe transfer must not continue.
24	True	Distress may communicate unmet need.
25	Protect/report	Unnecessary exposure is a safeguarding and dignity concern.
26	A	Objective records support continuity and accountability.
27	Escalate	Sudden change may signal serious concern.
28	True	Refusal and context may affect safety and review.
29	Downtime process	Emergency and care continuity come first; record accurately later.
30	Do not share	Personal-care images require strict authorisation.

31	Integrated safety	Choice is supported through safer alternatives.
32	B	Physical competence must be observed.
33	Remediate	Unsafe skill performance must not be signed off.
34	False	The credential is caregiver CPD, not a regulated clinical qualification.
35	Integrated response	Dignity, safety, safeguarding, documentation and review all apply.

18. Practical competency verification framework

Practical verification should be conducted by an appropriately qualified instructor/assessor using a safe environment, suitable equipment and the learner's current employer procedures. The assessor should observe preparation, communication, consent, privacy, technique, infection prevention, independence, monitoring, documentation and escalation.

Station	Demonstrated capability
Room entry and consent	Knocks, introduces self, explains, asks consent and provides choice
Personal-care preparation	Prepares equipment, privacy, temperature, environment and access
Bathing/bed bath	Sequences safely, maintains warmth/privacy and observes skin
Grooming/oral care	Supports preference, positioning, equipment hygiene and reporting
Dressing	Encourages choice and participation; assists without force
Toileting/continence	Maintains dignity, safe access, hygiene and discreet product use
Mealtime support	Hand hygiene, safe positioning, pacing, independence and escalation

Mobility/positioning	Uses only trained technique and equipment; no unsafe lifting
Adapted communication	Uses accessible communication for dementia/disability/sensory need
Documentation	Records assistance, observations, changes and escalation objectively

Critical-fail behaviours

Critical failure includes intimate care without consent or explanation; unnecessary exposure; unsafe transfer or lifting; ignoring dizziness, pain or breathing/swallowing concern; force-feeding; mocking or humiliating language; unauthorised clinical procedure; safeguarding concealment; or signing off a skill not demonstrated.

19. Practical competency checklist

Criterion	Pass / Remediate	Notes
Confirms identity and preference		
Explains task and seeks consent		
Protects privacy and body coverage		
Prepares equipment and environment		
Performs hand hygiene and equipment cleaning		
Assesses assistance level		
Promotes client participation		
Uses safe caregiver body mechanics		
Does not perform beyond training		
Monitors comfort, pain, dizziness and distress		

Observes and reports relevant changes		
Communicates adapted support		
Documents objectively		
Reviews and escalates appropriately		

20. Online CPD completion requirements

Complete all six modules, required learning activities and module assessments; achieve at least **80% (28/35)** on the final assessment; and acknowledge the scope statement. The online certificate recognises knowledge completion only and does not represent observed physical competence, nursing, therapy, medicine, licensure or statutory certification.

21. CPD + practical verification requirements

Meet all online requirements and successfully demonstrate the specified practical skills using the checklist in Section 19. The practical record should include learner identity, assessor identity and qualification, date, equipment, observed stations, result, remediation, limitations and review/reassessment date.

22. Certificate wording

Online CPD completion certificate

This certificate recognises successful completion of NIC online continuing professional development in activities of daily living assistance and personal care, including person-centred support, consent, dignity, privacy, assistance levels, bathing, grooming, oral care, dressing, toileting, continence, mealtime support, hydration awareness, mobility boundaries, adapted care, safeguarding, documentation and review. **This certificate represents online CPD knowledge completion only. It does not establish hands-on practical competence and does not qualify the holder as a nurse, physiotherapist, occupational therapist, medical practitioner or licensed clinical professional.**

CPD with verified practical competency

This certificate recognises completion of NIC online ADL and personal-care CPD and successful practical verification of the specific skills listed in the attached assessor record on [date] by [assessor/provider]. The endorsement represents demonstrated competence in the specified assessed activities at the time of assessment and does not constitute nursing, therapy, medical licensure or automatic statutory workplace certification.

23. Digital badge specifications

NIC ADL & Personal Care — Knowledge Badge

Field	Specification
Description	Recognises online CPD knowledge in safe, respectful, person-centred ADL support.
Competencies	ADL/IADL understanding, consent, dignity, assistance levels, personal care, continence, meals, mobility boundaries, adaptation, safeguarding and documentation
CPD hours	5–6 online hours
Evidence	Module completion, assessments, final score $\geq 80\%$, scope acknowledgement
Verification	NIC badge URL/QR, certificate ID, date and curriculum version
Limitation	Not practical competence, licensure or statutory certification
Review	Recommend review every 24 months or sooner after material guidance/policy change

NIC ADL & Personal Care — Practical Competency Verified Badge

Field	Specification
Description	Recognises online CPD plus observed verification of specified personal-care skills.
Competencies	Only the activities listed in the assessor record

CPD hours	5–6 online hours plus separately recorded practical assessment
Evidence	Online completion, assessor record, checklist, date, equipment and result
Verification	Badge URL/QR, assessor/provider, certificate ID and reassessment date
Limitation	Not nursing, therapy, medicine, licensure or automatic statutory certification
Review	Follow employer/provider requirements; recommend reassessment at least every 24 months unless sooner is required

24. NIC portal course description

Activities of Daily Living Assistance & Personal Care is a 5–6 hour NIC online CPD micro-credential for caregivers who provide direct support with everyday living.

Learners progress from understanding ADLs to person-centred care, consent, dignity, assistance levels, bathing, grooming, oral care, dressing, toileting, continence, meals, hydration, cultural preferences, mobility, adapted care, resistance, safeguarding, documentation and review.

The portal must display **Course → Modules → Lessons → Module Assessments → Final Assessment → Certificate** and must clearly distinguish online knowledge completion from any practical competency endorsement.

25. Visual asset plan

Asset	Purpose
ASPSIMDR pathway diagram	Show the complete decision framework
Assistance-level ladder	Illustrate independent to full assistance
Privacy room diagram	Show doors, curtains, body coverage and equipment placement
Bathing safety illustration	Show non-slip, temperature awareness and accessible supplies
Bed-bath sequence card	Support practical sequencing and warmth

Toileting dignity illustration	Show privacy, access and call method
Mealtime positioning graphic	Show safe, plan-led positioning without diagnosing swallowing
Mobility risk illustration	Show route, brakes, footwear and no unsafe lifting
Safeguarding boundary poster	Show appropriate and inappropriate professional behaviours
Documentation example	Compare objective and judgemental wording

All visuals should use diverse Nigerian adults, avoid humiliating exposure and display consent and privacy. Practical images should be reviewed by a caregiver educator and safeguarding lead before publication.

26. Downloadable job aids

1. **ADL Assistance Checklist:** Assess ability, ask preference, plan, support, promote independence, monitor, document and review.
2. **Personal Care Preparation Checklist:** Equipment, privacy, temperature, lighting, route, products, consent and infection prevention.
3. **Dignity & Privacy Checklist:** Knock, explain, consent, cover, close, respectful language, time and belongings.
4. **Bathing/Showering Safety Checklist:** Water, non-slip, access, fatigue, dizziness, skin observation and reporting.
5. **Toileting & Continence Support Guide:** Safe access, privacy, hygiene, products, skin awareness, hydration and reportable changes.
6. **Mealtime Assistance Checklist:** Hand hygiene, positioning, utensils, pace, preference, intake and swallowing concerns.
7. **Independence Support Planner:** What the client can do, desired help, prompts, equipment, risks and review date.
8. **ADL Observation & Documentation Template:** Activity, assistance level, participation, observations, actions, escalation and follow-up.
9. **Caregiver Practical Competency Checklist:** Section 19 stations and critical-fail behaviours.

27. References

- [1] World Health Organization, Integrated Care for Older People (ICOPE).
- [2] Social Care Institute for Excellence, Privacy and dignity in care.
- [3] Care Quality Commission, Regulation 10: Dignity and respect.
- [4] World Health Organization, Integrated care for older people: guidelines on community-level interventions to manage declines in intrinsic capacity.

Source-use statement: These sources support international professional principles. They do not replace Nigerian law, employer policy, care plans, safeguarding procedures, infection-prevention procedures, moving-and-handling training or specialist clinical guidance. NIC should obtain local governance review before publication.

28. Quality assurance review

Quality question	Finding	Action before launch
Practical value	Framework, scenarios and job aids are designed for home and facility care.	Pilot with working caregivers.
Dignity	Privacy, consent, body coverage and respectful language are embedded in every intimate-care lesson.	Safeguarding review of visuals.
Independence	Assistance ladder and “care with me” principle prevent unnecessary takeover.	Validate examples with clients/caregivers.
Consent	Consent, refusal, choice and communication difficulties are addressed.	Confirm local policy and capacity pathway.
Safety	Falls, water, heat, skin, infection, food, swallowing and transfer risks are included.	Cross-check with employer procedures.
Scope	Clinical, therapy, catheter, wound, swallowing and specialist transfer boundaries are explicit.	Governance sign-off.
Cultural relevance	Nigerian family, language, faith, fasting, food, water/power and shared-space realities are acknowledged.	Add local examples from pilot sites.

Safeguarding	Intimate-care boundaries, abuse indicators and reporting are included.	Confirm reporting contacts and escalation route.
Visual quality	A visual asset plan supports practical learning without humiliating exposure.	Commission accessible, diverse illustrations.
Non-duplication	Links to NIC' s First Aid, Gerontology, Chronic Care, Dementia, IPC, Mobility, Nutrition and Documentation courses are included.	Add portal cross-links.
Professional credibility	Separate online knowledge and practical verification pathways make employer meaning clear.	Approve assessor standards and certificate wording.

Final implementation checklist

Before publication, NIC should insert current local safeguarding contacts, employer escalation routes, infection-prevention procedures, transfer-equipment standards, specialist referral routes, privacy requirements, practical assessor qualifications and reassessment rules. The credential should be reviewed whenever relevant law, policy, clinical guidance or employer procedure changes.

The intended learner outcome is:

I know how to provide personal care professionally. I can support a person' s daily needs without taking away independence. I understand how to protect dignity, privacy and choice, recognise risks and changes, and provide safe assistance within my scope.